NIHR | National Institute for Health and Care Research

NIHR Doctoral Fellowship (DF)

Funding, Support or Development Opportunity	2025/303 NIHR Doctoral Award (Cohort1)

SUMMARY INFORMATION

Application Title	The Jewish MAMMA project: participatory research with the Orthodox Jewish community in the UK
Contracting Organisation	City St George's, University of London
Partner Organisation(s)	HOMERTON HEALTHCARE NHS FOUNDATION TRUST
Start Date	01/04/2026
End Date	31/03/2031
Duration (months)	60
% FTE Commitment	60%
Percentage of award time undertaking practice/clinical work	0%
Total Research Costs Requested	£548,855.00
Has this application been previously submitted to this or any other funding body?	No
Partnership Fellowship	Not Applicable

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LEAD APPLICANT & RESEARCH TEAM

Lead Applicant

NIHR Doctoral Fellowship (DF)

Full Name	Ms Michal Meroz
Organisation	City St George's, University of London
Department	Centre for Maternal and Child Health Research
Professional Background	Midwife
Professional Registration	Registered midwife, NMC, PIN 20H0529O Royal College of Nurses, 3884425 Registered nurse-midwife, Israel Ministry of Health, PIN 185109
ORCID	0009-0001-9294-6498
Research Degree Status	Not registered
Date of Registration	
Percentage FTE uptake of research degree	
Date of submission of research degree	
Date of award of research degree	
Contextual Factors	Since my early days as a nursing student in 2000, despite the personal challenges of being a young single parent, I have always strived for excellence. I was granted scholarships, achieved awards, and graduated with distinction. In 2018, I emigrated to the UK to pursue an academic career, as opportunities in Israel were limited. The experience of being a new immigrant, living far from family, friends, and any other form of support, and navigating a new healthcare system presented personal and professional challenges. This lived experience helped me appreciate the cultural and language barriers many people in the UK experience and has fuelled my motivation to explore inequalities in care. Despite being secular, I am familiar with Jewish traditions; the Orthodox community in the UK has confirmed that this makes me a trustworthy partner that people from this insular community are happy to work with. This is invaluable, as the literature and my groundwork have demonstrated the challenges that the health and public sectors experience when trying to engage with this community. I have always approached clinical care with an academic mindset, and while hoping to improve my research skills, I applied for a senior research midwife role a few years ago. Supporting numerous local and international studies has allowed me to gain extensive experience in applied research, including research and governance procedures, ethics, and understanding the barriers to research faced by local communities and NHS staff, as well as strategies to address these challenges. However, this role has not enabled me to foster my research profile as I had hoped, and I now feel ready to focus on my own research project and embark on my academic career. In recent years, I have been experiencing severe symptoms that have led to an ongoing medical investigation. After careful consideration, I decided to apply to the programme on a part-time basis, allowing time for appointments and self-care.

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APPLICATION DETAILS

Scientific Abstract
Background
The Orthodox Jewish (OJ) are an observant religious community with unique health, cultural, and religious needs that are not always met by the NHS. Despite their heavy reliance on maternity services due to remarkably high fertility rates of seven children per woman amongst several subgroups, OJ families are largely absent from UK health research and policy, and a recent NHS report highlighted that these communities have been overlooked. Strict religious laws, insular lifestyle, language and cultural barriers and digital exclusion can create interaction challenges. This research aims to fill gaps in knowledge and co-produce the Jewish MAMMA card, to aid these interactions
Research Questions
1. What information do OJ families wish to include in a MAMMA card, and what is their preferred design? In particular:
A. What are the unique needs and experiences of OJ women and their partners when interacting with NHS maternity services, and how are these currently being met by NHS services?
B. What can be done to increase the sense of belonging and ownership of Orthodox Jewish families to NHS maternity health promotion programmes?
2. What features and/or health promotion do NHS maternity staff wish to include in the MAMMA card?
3. What are the challenges and enablers that Orthodox Jewish women and NHS maternity staff face when working together, and how can these be addressed using the MAMMA card?
Aims and Objectives
The overall aim of the proposed study is to co-produce a communication tool, the Jewish MAMMA Card, that will enhance mutual understanding and respect between OJ communities and NHS maternity staff.
Methods
The proposed study is participatory research, consisting of three work packages (WP):
WP1: Ethnographic fieldwork, including observations in maternity settings, interviews and focus groups with Orthodox partners and NHS staff across London, Manchester, and Gateshead.
WP2: Two co-design workshops will bring together Orthodox women and NHS staff to collaboratively draft content for the MAMMA Card.
WP3: Final design of the Jewish MAMMA Card by a community graphic designer, with input from research and PPIE advisory groups.
The project will be informed by Intersectionality theory, Douglas's model of Purity and Danger, the Positive Deviance Model and Freire's Pedagogy of the Oppressed.
Working with People and Communities (PPIE)
Strong public involvement was established during the applicant's Pre-doctoral Clinical Academic Fellowship, including trust-building with Orthodox communities and early testing of project feasibility. A PPIE group of Orthodox women and stakeholders will be embedded in every stage. Community voices will shape recruitment, interview guides, workshop structure, and card content.
Implications for Practice
The MAMMA Card will bridge families and NHS staff, fostering person-centred care, cultural safety, and reduced miscommunication. The model will be replicable for other ethnic minority communities and adaptable across NHS maternity settings.
Dissemination of Findings
Creative dissemination will include photographic storytelling, undertaken by the project's PPIE lead. In addition, paper-based materials will be distributed locally to OJ communities, and academic papers will be published. Via a reflective approach, the main applicant will also write and publish notes from her experience of working with the community. The MAMMA Card may be piloted as part of a future NHS quality improvement project.

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Plain English Summary

The Jewish MAMMA (My Authentic Me in MAternity) project

What is the Jewish MAMMA project?

The Jewish MAMMA is a research project that will help people working in maternity (mother and baby units) understand Orthodox people better. This will make Orthodox Jewish couples feel safer and supported when they are pregnant and having their baby

Why do we need it?

Orthodox Jews are a strict religious group with unique cultural needs and customs. Some Orthodox people speak only Yiddish or Hebrew. Orthodox couples sometimes feel that NHS staff do not understand what they need during pregnancy and childbirth. For example, NHS staff sometimes book their appointments on Jewish holidays, when Orthodox Jews are not allowed to travel, write or use electricity and phones. To make things better, people from the community wanted to design a new pregnancy card. They want the card to have information about Jewish customs to help NHS staff give them better care. People from the NHS wanted the card to also include specific information and recommendations for pregnancy and childbirth.

What will we do during the project?

We will create the Jewish MAMMA card, with information in English and Yiddish. In the first stage, I will talk to Orthodox women and ask them what they need when they are pregnant and when they deliver their babies. Orthodox men can join a separate group, which an Orthodox man from Manchester will lead, to explain their needs. Everything people say will stay anonymous, which means that we will delete all participants' details, and no one will know who took part.

I will also meet with NHS maternity staff to understand what is important to them when they care for Orthodox couples, and what they can do to give them a better childbirth experience.

The second stage will be workshops. NHS maternity staff and Orthodox women will think together of ideas for the Jewish MAMMA Card.

In the third and last stage, I will work with people from the community to design the final card and print a few copies.

After the project, NHS staff will try the card, and it will then be available to everyone in the community.

Who can join the project?

Orthodox Jewish adults and NHS maternity staff.

How will community members be involved?

Orthodox people will be involved in all stages of the project. They will ask their friends to join, lead workshops, and share their experiences with other people from the community. Mrs. Tova Schprecher, an Orthodox mother and a photographer, agreed to join me on the project. She speaks English, Yiddish and Hebrew. She will take pictures during the workshops and create posters and a presentation. Other people from the community, for example, people working in Orthodox charities, also agreed to join and help.

Where will the project be?

In London, Manchester and Canvey Island, where most Orthodox Jews in the UK live.

How long is the project?

This is part of a five-year fellowship.

Where will we share information about the project?

We will advertise the project in local newspapers to keep the community involved. I will also write academic papers and publish in journals. Mrs. Schprecher will publish her pictures in community newspapers and will present her work at academic conferences. We will also create posters to put up in maternity units and community centres.

Who will support the project?

Tahareinu, a charity that helps Orthodox people during pregnancy and childbirth. Homerton, Whittington and St Mary's hospitals will also support

Working with People and Communities Summary

The involvement of people from the Orthodox Jewish (OJ) community is at the core of my project. During my PCAF, I established rapport and trust with people and charities that will help with the snowball sampling, community engagement and specific tasks. Segments of the application were

assessed by PPIE members before submission. I contacted an Orthodox man who will facilitate focus groups with men during WP1. I identified venues in the community for the WP2 workshops.
In addition, I have learnt that:
• Word of mouth and local newspapers are the most trustworthy sources of information in the community and the best way to recruit and disseminate findings
• There is no cultural competence within the NHS regarding the Orthodox community and their unique needs. The design of a sustainable tool was perceived as more realistic than the study days that had previously failed
• OJ people regularly seek maternal health advice. However, many prefer rabbis or private doctors as they find the NHS unreliable and not aligned with their needs
• I should focus on the strengths rather than barriers, which led me to change the initial name and to adopt the Positive Deviance (PD) approach (see Detailed Plan)
• Information should be available in Yiddish/Hebrew, and as paper print
During the next stages lived experience of interviewers from WP1 will inform WP2. Orthodox women will help recruit, sit in the advisory group, keep the community engaged, help interpret and disseminate findings and read research material.
In addition, specific tasks were agreed upon during my PCAF (all are Orthodox and key figures in their communities):
1. Tova Schprecher: PPIE lead, photographer and certified Yiddish medical translator. Help design culturally appropriate materials for the community (PISs, consent forms etc), including translation to Yiddish; lead the PPIE group and co-facilitate meetings; keep community engaged and recruitment targets; support during WP2 workshops, including interpretation and creation of a photographic storytelling; create short videos to display in community; present at RCM conference; review segments of papers for publications (e.g., plain English summary). On her support letter, Mrs. Schprecher wrote: “having witnessed the communication challenges between our community and NHS services firsthand, I’m particularly motivated to help create this practical tool.” I have already started working with Mrs. Schprecher during my PCAF and we communicate regularly, discussing our future collaboration.
2. Rachel Klien, founder and director of HJMT doula charity: London PPIE lead, facilitate WP2 workshops
3. Rachel Jacobs, midwife, Manchester NHS staff lead. Facilitate WP2 workshops
4. Sarah Barnett, midwife and an Orthodox fertility charity volunteer, London NHS staff lead. Facilitate WP2 workshops
5. Devora Roberts, graphic designer: join the workshops in WP2, design MAMMA card
6. Marian Rabinowitz, Homerton’s patient advocate: support with recruitment
7. Ms. Ora Jesner, obstetric consultant: review the MAMMA card to assess for medical accuracy
8. Mr. Johnny Weinberg: facilitate men’s focus groups
9. Tahareinu, a global charity addressing women's health challenges in the Orthodox Jewish community through medical and Halachic guidance. From Rabbi Melber, director: “We’re pleased to support Michal’s MAMMA project with Halachic advice, community connections, and a visit to our Israeli hub, which fields over 1,000 weekly hotline calls”

SUPPORT AND MENTORSHIP

NIHR Doctoral Fellowship (DF)

Full Name	Dr Kylie Watson
Organisation	MANCHESTER UNIVERSITY NHS FOUNDATION TRUST
Email Address	kylie.watson@mft.nhs.uk
Role	Mentor
Description of Support Provided	As a Consultant Midwife at Manchester University NHS Foundation Trust, Dr Kylie Watson represents a role I aspire to on my journey toward becoming an internationally recognised academic leader in midwifery research. Dr. Watson's unique combination of clinical and academic expertise makes her an exceptional mentor. Originally trained in New Zealand, she brings an international perspective and vast clinical experience across diverse healthcare settings. Dr Watson completed her NIHR ICA Doctoral Fellowship at the University of Manchester, studying wireless telemetry in fetal heart rate monitoring. Dr Watson has also authored a chapter in Mayes Midwifery, which is a highly influential textbook in midwifery, and she is an author on several peer-reviewed academic papers. Dr Watson's recent research has focused on inequalities and maternity and aligns closely with my interests in this area. Dr Watson has experience of mentoring NIHR Predoctoral candidates and PhD students, and mentors a number of midwives within her own organisation, contributing to capacity and capability building for midwifery research career pathways. Dr. Watson has committed to quarterly one-to-one sessions and opportunities to work alongside her team throughout the award period. These sessions will focus on: • Career development strategies for academic leadership in midwifery research • Balancing research with clinical practice • Developing leadership skills in maternity care • Maintaining 20% clinical time during research activities • Building professional networks is essential for future research funding This structured approach provides regular reflection opportunities while maintaining flexibility to address emerging challenges.

Full Name	Dr Christine McCourt
Organisation	City, University of London
Email Address	Christine.McCourt.1@city.ac.uk
Role	Primary Academic Supervisor
Description of Support Provided	Professor Christine McCourt is an experienced researcher and academic with a specific interest in maternity. She has been involved in and led numerous studies in the UK maternity sector, which have directly led to national policy changes. Her years of involvement with place of birth research, particularly as part of the Birthplace in England collaborative group, have informed national guidance on caring for and advising women on choosing a place of birth in the UK. She subsequently conducted studies focused on implementing national policy and guidance based on these findings and has undertaken evaluations of the implementation of evidence-based policy and practice.

Professor McCourt also has significant knowledge and experience in designing and managing complex intervention research and ethnographic studies of healthcare. She has also studied relationships of maternity professionals' and service users' experiences of care and how these relate to evidence and policy. She has written extensively about social and anthropological theory as it applies to maternity practice in the UK and globally and has expertise in relevant theory and methods for this subject, which has specific relevance to my research topic. She has conducted a number of studies and published peer-reviewed articles focused on equity of access to care, care experiences and outcomes for people from minority ethnic communities, and her interest in the topic is of high significance to my study.
Professor McCourt has considerable supervision experience as well as experience in mentoring less experienced supervisors and postdoctoral researchers. Her supervision experience includes a record of over 30 Master's and 31 PhD students who were successfully supervised to completion. Her record of conversion of Master's to PhD students is four; she is currently supervising six PhD students as an internal and three as external lead supervisor, one has recently had their viva. She has been an external examiner for 28 PhD, 1 MPhil and 1 MRes students.
Professor McCourt has been very responsive and supportive during my PCAF. We have had frequent Teams meetings and continuous email exchange, and she usually responded on the same day, which was very helpful in keeping the workflow. On a shared Teams folder, I kept all my project's documents, which I kept updating. I also created a live Gantt chart, which was kept in the same folder. Via this platform, we kept a communication flow and transparency of my work and progress. Professor McCourt was also an invaluable resource for courses and conferences to support my progression. In addition, she helped me create a City St George's student account, which gave me access to all university facilities and the library; this was extremely helpful, and I utilised the library services for short courses and librarian support for writing my papers and using a referencing system, RefWorks.
This arrangement will be of high value during a DF programme and will allow me access to high-quality research facilities and activities, including weekly research seminars, research group monthly meetings and selected project management meetings to gain understanding of research project management. Additionally, City St George's University offers a variety of courses to its students, including research methods and neurodiversity, which I intend to take. I also agreed on a few weeks' placement with the R&D department. Professor McCourt has committed to supporting me during the DF programme through regular monthly meetings and additional meetings or other forms of contact as needed. Her well-recognised position at the City St George's University of London, where she leads the Centre for Maternal and Child Health Research and her excellent external research network will widen my research work and allow me to further develop my own network.
Professor McCourt's experience and expertise in Maternal and Child health research and student supervision make her an excellent primary supervisor for my PhD, should I be successful. Her extensive knowledge of the UK maternity system as well as global maternal health issues, puts her in the best position to support doctoral work of high academic quality that is also relevant to the specific service users

	targeted by the proposal. Her impressive and ongoing academic career also means that she will be able to support me with academic writing and publication, in due time.

Full Name	Professor Carol Rivas
Organisation	University College London
Email Address	c.rivas@ucl.ac.uk
Role	Academic Supervisor
Description of Support Provided	Carol Rivas is Professor of Health and Social Care at University College London. Her work explores the interactions of the various stakeholders in organisational encounters, primarily health and social care systems, with a focus on marginalised groups and intersectionalities. Within this, she draws on sensitivities developed from her own lived experience. Carol is a mixed methodologist, with expertise in approaches ranging from machine learning and big data analysis to multi-modal ethnography. As such, she specialises in innovative transdisciplinary, participatory and arts-based methods, knowledge exchange and transformative outputs. She has recently been specialising in work that involves community co-researchers. She is co-author of the textbook Researching Society and Culture (5th ed) and authored a methods module/e-textbook for the academic publisher SAGE on culturally sensitive research. Professor Rivas' particular interests fit well with my proposed research, and her extensive knowledge has the potential to strengthen my understanding on marginalised groups and research methodologies that are most suitable to explore these communities. Her ethnographic expertise is of high value to my work. Professor Rivas has led several major studies, each involving the management of 8-12 researchers, most of whom went on to career advancement as a result of her mentorship. She was formal mentor on the NIHR-funded HEAL-D senior fellowship; her work on this won a national Quality Impact award and the HEAL-D intervention is now in clinical trials. She is also currently mentoring two research groups funded by the MS Society and by NIHR to enhance the authentic use of community-co-researchers. She has supervised numerous MSc and BSc dissertations, with 8 PhD students to completion since 2016, and currently supervising 9 PhD students, 2 of whom are in write-up and two of whom are part-time. Professor Rivas specialises in taking on students from marginalised groups as well as those researching topics relevant to marginalised groups. I have worked with Professor Rivas during my PCAF and found that her knowledge and experience contributed to the quality and depth of my work. as with the PCAF, we intend to keep an open communication channel vie emails, phone calls and in person meetings. We will also share a Teams drive to allow review and of my work and mentoring my progress.

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Full Name	Professor Anat Gesser-Edelsburg
Organisation	University of Haifa
Email Address	ageser@univ.haifa.ac.il
Role	Academic Supervisor
Description of Support Provided	Professor Anat Gesser-Edelsburg is the Head of the School of Public Health at the University of Haifa, Israel. Israel is home to the largest Orthodox Jewish community in the world and Haifa maintains twinning relationship with Hackney council since 1968. Her areas of expertise include health communication, risk communication, social marketing, and edutainment, with a particular emphasis on culturally sensitive contexts. Her work emphasises the importance of trust-building within insular communities and the co-design of health strategies that are both effective and culturally appropriate. Additionally, Professor Gesser-Edelsburg is a leading researcher in the Positive Deviance (PD) approach. She has pioneered its application in public health communication in Israel, identifying hidden solutions within communities that succeed against the odds. Her work in PD focuses on recognising and amplifying successful behaviours that emerge organically within communities, providing culturally sensitive solutions to health challenges. This innovative approach has proven effective in enhancing health outcomes by leveraging community strengths and local knowledge. I intend to adapt this approach in my work. Professor Gesser-Edelsburg's expertise in community-based participatory research and narrative inquiry and her experience in designing health communication interventions that emphasise cultural and religious values for improving health outcomes, aligns closely with the proposed study that focuses on Orthodox Jewish women and their unique cultural, religious and health needs. As a mentor and supervisor, Professor Gesser-Edelsburg has successfully supervised seven PhD students to completion. She is currently supervising several PhD candidates in public health. Her mentorship emphasises bridging gaps in health communication and promoting inclusive, community-tailored healthcare solutions. Her leadership in culturally adapted health communication and risk communication is not only theory-driven but also practically grounded in community-led interventions. Should I receive the award we intend to keep an open communication channel via email and a shared Teams folder that the whole study team will have access to. We will also have biannual team meetings and I will meet Prof Gesser-Edelsburg in person twice, during my visits to Israel. Prof. Gesser-Edelsburg's location in Israel and her contact with local community, stakeholders and researchers, is invaluable to my study.

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Full Name	Dr Marina Daniele
Organisation	City St George's, University of London
Email Address	marina.daniele@citystgeorges.ac.uk
Role	Academic Supervisor
Description of Support Provided	Dr. Marina Daniele has been a Midwifery lecturer at City St George's for 3.5 years. Marina holds degrees in midwifery, social sciences and epidemiology. Her career of 15 years spans clinical practice, research and teaching, working across a range of countries and settings. Marina's PhD was a mixed-methods project focused on male partner support for pregnant women and their inclusion in clinical care in Burkina Faso (West Africa). Marina's expertise on gender roles and cultural safety in childbirth will be an asset to my project, as there are related issues to explore in the case of the Orthodox communities. Most of Dr Daniele's current research is on maternal health inequalities related to perinatal care in England. She has the role of inequalities theme lead in the Maternal & Child Health Research Centre at City St George's. Marina has published on the accessibility of midwife-led birthing care for racialised birthing people. Social justice is a key element of her ethos as an academic. She has expertise in participatory methods and in the principles and philosophical underpinnings of conducting solidarity research with underserved communities. Marina is about to commence an NIHR Pre-Application fellowship to support her in developing an Advanced Fellowship application on equity in midwifery and perinatal health. She is thus on track to become an established researcher in this field. As such, there are significant overlaps in interest that match well with my profile and ambitions, as expressed in this application. Dr Daniele is currently a secondary PhD supervisor for four students who are at different stages of their PhDs. She has undergone PhD supervision training and is working closely with more senior colleagues to develop her skills further. She is also supervising six Master's dissertations and has supervised nine students to completion. Her support has been key to enabling two recent Master's graduates to publish their work in established peer-reviewed journals. Students describe her as a kind, engaged and highly responsive supervisor. I have had several discussions with Dr Daniele over the past weeks, and we agreed that she will join my supervisory team. During the DF, we agreed on Teams meetings once per term, with ad hoc meetings as needed. Dr Daniele will have access to our shared Teams folder and will be able to track my progress and review drafts.

SUMMARY BUDGET

Justification of Costs

Other staff

I included one day a month for PPIE lead costs (£3600 per year), including designing culturally sensitive materials, continuous consultancy, community engagement and support with knowledge mobilisation.

Travel and subsistence

Journeys were calculated based on car drives and train/bus rides from the rural area where I live. Long-distance stays included Holiday Inn and subsistence. My research population might have low digital

literacy, and I will travel to and stay a few weeks a year in Manchester, London and Canvey Island for recruitment, interviews, workshops and meetings

Equipment and consumables

An encrypted Dictaphone and a laptop to store data safely while travelling. I will also need stationery and prints. I included logoed pens to enhance the sense of connection to the project.

Knowledge mobilisation

Included are annual advertisements in community newspapers (prices estimated as per NIHR guidance) and posters in the NHS and community, PPIE lead work on her photographic storytelling, which we agreed on during my PCAF, including designing posters and videos and publication and presentation of these. I included a flight to Israel to present at the World Congress of Jewish Studies, Jerusalem.

Training and development

Halacha (Jewish laws) course was included to help me understand the community and to improve my interpretation and analysis of findings, priced at £250. I will undertake a few courses to enhance my research skills, including participatory research, community engagement, qualitative analysis, UK EQUATOR school course and NVivo. I also included the SAPPHIRE ethnography summer school, recommended by my supervisor. I will attend an interview skills course with the PPIE lead to support running the workshops, producing rich data and dealing with challenges. Members of my Academic Midwives Network recommended some of these courses.

I will attend a few relevant conferences, including the RCM conference, which will keep me up to date with research in the field while maintaining my midwifery registration. Additionally, the PPIE lead will present in year 5. I will join the Participatory Research conference that I attended during my PCAF, and found it relevant to my project. At the Congress for Jewish Studies, I will present, meet other researchers who focus on Judaism, and stay current with recent research in the field. Costs include two flights to Israel and accommodation during the congress. I will join the Race, Racism, and Health and the Social Research Association (SRA) conferences that are relevant to my research topic and were recommended by members of my Network. Fees for SRA and World Jewish Studies annual memberships are included. During my PCAF I identified and arranged placements in The Institute for Jewish Research in London to learn how to collect data on Jewish population, that is not routinely collected; St Mary's hospital to shadow my mentor; the National Perinatal Epidemiology Unit in Oxford to develop my career; and Homerton and City St Georges R&D on flexible days. I included journey and accommodation costs. I will also spend 5 months in Israel (self-funded). During my stay, I will meet my external supervisor and stakeholders from Israel, with experience in working with and communicating health messages to the Orthodox community, learn Halacha and shadow policy makers

Inclusion

Translation of patients' information £0.2 per word, and interpreter to Yiddish and Hebrew £60/hr were included. Neurodiversity in Academia course, to ensure the participation of neurodiverse individuals.

PPIE

People from the Orthodox Jewish community are my partners and are at the heart of this project. I will support them with training and specific tasks, reimburse them for study costs, journeys and accommodation, and time for participating in the project, consistent with NIHR guidelines. PPIE members will also help recruit, design, prepare and facilitate focus groups; design the MAMMA card; help disseminate findings within the community; and review study material. Costs were calculated as per NIHR guidance. Orthodox men are unlikely to speak freely to me, and during my PCAF, I identified a facilitator for the focus groups, calculated as £300/FG * 2. £10 per referral is included for snowball sampling. I arranged a placement at the Tahareinu charity, £250 annual donation was included for their support and consultancy throughout.

I included £300 per day*4 HCP and PPIE workshops facilitators. These people have already been identified and have agreed to contribute. Two PPIE leads will also attend focus group training.

Other direct costs

Workshops will be held in community facilities that are easily accessible, and kosher food will be served. I priced £20 per head as kosher is more expensive, and workshops will take half a day each. Community venues were identified by PPIE and hire costs were included.

Student fees, vouchers for participants, and childcare costs for 10 women during workshops.

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ACKNOWLEDGEMENT AND CONFLICTS

I have read and understood the terms of applying as Lead Applicant and accept this role	✓

Potential Conflicts	N/A

Lead Applicant:	Michal Meroz	**Application Number:**	NIHR503654
Contracting Organisation:	City St George's, University of London		
Proposed Start Date:	01/04/2026	**End Date:**	31/03/2031

	Year 1	Year 2	Year 3	Year 4	Year 5	Total Cost to NIHR
STAFF COSTS						
Lead and co-applicants	£46,151.00	£47,057.00	£47,057.00	£48,154.00	£48,154.00	**£236,573.00**
Research staff	£0.00	£0.00	£0.00	£0.00	£0.00	**£0.00**
Other staff	£3,600.00	£3,600.00	£3,600.00	£3,600.00	£3,600.00	**£18,000.00**
Total staff costs	**£49,751.00**	**£50,657.00**	**£50,657.00**	**£51,754.00**	**£51,754.00**	**£254,573.00**
OTHER RESEARCH/ AWARD COSTS						
Travel and subsistence	£1,815.00	£4,602.00	£2,941.00	£1,516.00	£2,168.00	**£13,042.00**
Equipment	£1,270.00	£0.00	£0.00	£0.00	£0.00	**£1,270.00**
Consumables	£15.00	£125.00	£0.00	£380.00	£0.00	**£520.00**
Knowledge mobilisation, dissemination and impact	£200.00	£200.00	£200.00	£200.00	£1,250.00	**£2,050.00**
Training and development	£1,300.00	£2,140.00	£544.00	£185.00	£1,004.00	**£5,173.00**
Research inclusion	£1,500.00	£1,200.00	£0.00	£120.00	£0.00	**£2,820.00**
Working with people and communities	£1,130.00	£3,198.00	£2,568.00	£8,100.00	£4,940.00	**£19,936.00**
Other direct costs	£3,115.00	£6,615.00	£3,115.00	£10,706.00	£3,115.00	**£26,666.00**
HEI indirect costs	£44,111.00	£44,111.00	£44,111.00	£44,111.00	£44,111.00	**£220,555.00**
Other partner organisation indirect costs	£250.00	£500.00	£500.00	£500.00	£500.00	**£2,250.00**
TOTAL COST TO NIHR	**£104,457.00**	**£113,348.00**	**£104,636.00**	**£117,572.00**	**£108,842.00**	**£548,855.00**

Co-funding from other sources						**£0.00**
Cost to NIHR						**£548,855.00**
NHS support costs	£110.00	£110.00	£110.00	£110.00	£0.00	**£440.00**
NHS excess treatment costs	£0.00	£0.00	£0.00	£0.00	£0.00	**£0.00**
Non-NHS excess treatment costs	£0.00	£0.00	£0.00	£0.00	£0.00	**£0.00**
TOTAL FUNDING						

Application Budget: NIHR503654

REQUESTED	£104,567.00	£113,458.00	£104,746.00	£117,682.00	£108,842.00	**£549,295.00**

Narrative CV template for Personal Career Development Awards

****Lead Applicant / Co-applicant Name:****
Michal Rosie Meroz

****Module 1 – Contributions to the generation of new ideas, tools, methodologies or knowledge****
In 2000, I started my nursing studies, during which I was awarded annual scholarships based on my academic achievements and social status (single parent). I started working in one of the largest Neonatal Intensive Care Units in the country, providing care to severely ill babies from Israel and Gaza. In addition to my main role, I created and delivered educational sessions for parents, introducing them to the unit and medical jargon, engaging them in their babies' care. I also helped maintain the breastmilk facilities, encouraging pregnant people to maintain their milk supply. During this time, I was a research nurse coordinator for a small-scale study. In my following role as a nursery nurse, my passion for breastfeeding led my manager to allocate me supernumerary hours to educate and support pregnant people with breastfeeding challenges, which later became a mandatory requirement in all Israeli hospitals. In 2012, my master's dissertation investigated the social and institutional perceptions of homebirths. The dissertation was rated as one of the top dissertations. I was awarded a prize from the Authority of Advanced Studies and presented in a student conference at Haifa University, at the Human Rights at Birth Conference, and at the Myers JDC Brookdale Institute Conference. Based on the dissertation and with the support of my supervisor, I published an academic paper. In 2014, I established a collaboration with the E.M.M.S. hospital in Nazareth to support families that opted for midwifery care, which was not available in Israeli hospitals. I was given a private room that, over time, became my private midwifery unit. During the same period, as a core member of the Israeli Homebirth Midwives Association, I contributed to new ideas and ways to support pregnant people with their place-of-birth decisions. I also provided tailored, holistic antenatal educational sessions to families and delivered many babies in different settings. While volunteering with the Israeli ambulance services, I identified the need for paramedics to be better trained on obstetric emergencies, and I designed and delivered an Obstetric Emergencies course. I later replicated this to create a new course at Zefat College. Utilising my professional knowledge, I established "Stork" at my home community, a volunteering service supporting families during the immediate postpartum period. On one occasion, we had a single-sex couple with preterm triplets; I coordinated a complex six-week support including meals, feeding, and babies' care; the success of the project was shared with the community via local meetings and newsletter. While studying Naturopathy, I provided free sessions to people from my community and delivered a Healthy Food course to promote a healthy lifestyle. I was also a first aid volunteer on the village's Crisis Mission Force.
I emigrated to the UK in 2018, and while studying for my MSc, I undertook many short courses at the University of Oxford, including Stata, Excel, design, EndNote and writing skills. As part of my work as an audit midwife at Whipps Cross Hospital, I designed an infographic for staff and families, presenting the audit's findings as leaflets and posters, hung in every birth centre room. At Homerton, I implemented a few initiatives to promote research within the trust, including a 30-minute monthly lecture to all midwives during mandatory training, incorporating interactive learning methods (quiz); elective placement for student midwives with the research team; and friendly competitions to encourage clinical teams to refer eligible women to our team.

****Module 2 – Contributions to the development of others and maintenance of effective working relationships****
I have an extensive record of teaching and mentoring, as a lecturer and simulation lead in Israel, I have mentored hundreds of nurses and always had the highest rated feedback from students. I have created many obstetrics and gynaecology simulations that I passed on to

my successors. At Homerton, I provide supervision and mentorship to students, new staff, midwives and junior doctors. I have been the line manager of 11 staff members so far, and two have successfully enrolled in band 7 roles. I work closely with my team, lead by example and provide continuous feedback and annual appraisals. As part of my NIHR Principal Investigator Pipeline Programme, I am mentoring a paediatrician registrar under the Associate PI Scheme. I sit on management joint meetings, represent the research team in departmental forums, and promote study findings across the service. I create annual infographics to present our studies and recruitments with the department. In June 2025, I completed the NIHR PCAF programme. These days, I am discussing ways to share my experience of the programme at Homerton trust.

I completed two formal leadership programmes and Tricky Conversation training to enhance my knowledge in strategic leadership, team dynamics and conflict resolution. I apply these skills daily, whether mentoring and supporting staff or resolving conflicts. I am a member of a few Jewish, midwifery and academic network groups, some of which I have joined during my PCAF. Recently, I have founded the Academic Midwives Network. During my PCAF, I utilised these platforms to engage people for PPIE activities, meet key figures from the community and identify relevant courses for my PhD.

I maintained my Israeli professional registration and have a large network in the country, where the largest Orthodox Jewish community in the world resides. I also maintained professional connections with Prof. Gesser-Edelsburg, my Master's supervisor, who has since become the head of public school at the University of Haifa and is keen to join the MAMMA project as an external supervisor. As part of my groundwork for the PhD project, I established connections with the Orthodox Jewish communities in London and Manchester, and I found the community to be receptive to the project and me. I am in continuous communication with the project's PPIE lead, and we agreed on a sub-study, a photographic storytelling project that she will be leading, with my support.

****Module 3 – Contributions to the wider research community****

My commitment to advancing knowledge in maternal health through evidence-based inquiry has led me to undertake the Evidence-Based-Health-Care programme at the University of Oxford. I achieved 95% on my last assignment grade, and the feedback stated: "This was an outstanding assignment, one of the best I have marked." My dissertation employed a national survey to explore emergency preparedness across UK midwifery units, identifying gaps and opportunities for standardisation. Supported by my supervisors, I published an academic paper based on this work. I regularly attend professional conferences and research study days. I communicate with the chief investigators of leading national and international maternity studies through Teams and face-to-face meetings, and emails. I recruit and conduct studies' follow-ups, while adhering to Good Clinical Practice (GCP) standards. I have completed over 250 hours of professional development training in the last two years, including cultural competence, quality improvement, remote consent, PIPP and public involvement courses, all of which reinforce my capacity to contribute to an inclusive, safe, and high-quality research environment. I refresh my GCP certificate every two years. I am the principal investigator (PI) for a few studies, including several that focus on inclusion, including Legacies and Future and the REPRESENT study, ensuring research and care are accessible to the diverse UK population. During my PCAF, I have submitted two papers for publication, and I am in the process of writing a scoping review of the evidence concerning the unique needs and experiences of Orthodox Jewish women and their partners using maternity services in the UK. This will enhance my knowledge on the topic and help identify gaps that need addressing. I was also asked to review a paper for PlosOne, which I accepted; this gave me the confidence to review papers in the future.

****Module 4 – Contributions to broader society****

I have dedicated my career to advancing culturally sensitive, inclusive, and equitable maternity care, while keeping local communities engaged. I have worked in Israel, Uganda

and the UK in all birth settings, from hospitals, through birth centres to community and home settings. In all three countries, I worked with diverse communities, including Muslim, Jewish and pregnant people from rural developing countries. During my work as a midwife in Nazareth, I joined the Midwives of Peace, a Palestinian Israeli association that aims to bridge political barriers through passion for midwifery. In 2013, I took a two-month career break and travelled to Uganda to volunteer as a senior midwife in a rural birth centre, delivering care and education to local pregnant people and exchanging knowledge with local midwives. By engaging people in Israel, I collected a large amount of donations that I took with me and was accepted warmly by local communities. I kept an open online blog of my journey and later presented the project at the Israeli Association conference. Since my early career, I was always keen on involving people and communities in their care, through empowerment, education and collaborations. I am passionate about promoting health and presenting research findings in a friendly way, which can be demonstrated on my application's infographics. I am also passionate about inclusion in research and as part of my current role, I have ongoing discussions with trial teams to make sure studies are accessible to our communities; for example, I provide trial teams with our local communities' five most common languages and encourage them to provide study materials in these languages.

During my PCAF, I gave a talk at a City and Hackney Council team meeting, I conducted drop-in sessions to inform NHS staff of my research project, and I was recently invited to provide a workshop to student midwives at City University on the topic of diversity. I am an advocate of personal choice and informed consent, and I promote people's right to choose their place of birth by sharing my knowledge on birth models that work. For example, in April 2022, I was invited to present to an Israeli committee that was appointed by the government to discuss birthplace options and re-visit the closure of midwifery-led units a few years earlier, I was proud to present the UK midwifery-care model, that is patient-centred and very different to the Israeli medical model. I was recently asked by the Israeli Midwives Association to disseminate the findings of my future research, once they are available. During my PhD, I intend to support people from the community to engage in research and disseminate findings within the community.

****Module 5 - Your suitability for this award****

This doctoral award is a natural progression of my professional mission: to contribute to safe, equitable, and person-centred maternity care that reflects the diversity of the NHS. I have always worked with minority and marginalised communities in different countries, striving to improve health outcomes and enhance communication between service users and care providers. My broad clinical experience, from community midwifery and birth centres to hospital-based care, demonstrates a unique breadth. With over 10 years of teaching and extensive leadership training, I have demonstrated my ability to guide and influence others. I aim to utilise everything I have achieved and learned so far and become a researcher, bringing together my clinical and academic knowledge and experience. The award will allow me to dedicate time to my passion.

During my PCAF, I established rapport with one of the most insular communities in the UK, that have been overlooked for many generations. This required hard work and demonstrates sensitivity and dedication. I have also established professional relationships with academics and NHS staff, and I feel like I have a well-established support network that is so important for professional development. My proposed doctoral research with the Orthodox Jewish community is in line with the NIHR priorities to reduce health inequalities and include underserved populations in research and policies. My dual Israeli and British nationality, having lived in both countries and being bilingual, places me in a unique position to navigate between the NHS and the Jewish community's needs. The extensive groundwork I have completed during my PCAF, including very positive feedback from the Orthodox Jewish community, reassures the success of the project.

The Jewish MAMMA (My Authentic Me in MAternity) project: participatory research with the Orthodox Jewish community in the UK.

****Background and rationale****

The United Kingdom (UK) is home to an increasingly diverse population, and according to the 2021 Census, it is now more ethnically diverse than ever before (Office for National Statistics, 2022). Within this multicultural society, the Jewish community represents a small ethnic group of approximately 300,000 people, 19% of whom identify as Orthodox (Graham and Boyd, 2024). This cultural diversity may introduce challenges to NHS maternity staff in terms of delivering equitable and culturally appropriate care. Local reports continue to indicate that women of ethnic minority backgrounds are at higher risk of developing pregnancy and childbirth complications, including maternal and neonatal deaths (NHS England, 2022; Knight et al., 2023; Kapadia et al., 2022). While a growing body of literature focuses on inequalities concerning Black and Asian communities, research involving smaller minority groups, such as Gypsy, Roma and Traveller, asylum seeking, and Orthodox Jewish people is scarce. Lack of understanding concerning the religious practices and cultural needs of these communities may exacerbate the already higher risk shared by all ethnic minority groups (McKnight et al., 2019; Hollinshead et al., 2023; NHS Race Health Observatory, 2024). The proposed study will focus on the Orthodox Jewish community, a group that has *"often been neglected and overlooked in conversations about health inequality"* (NHS Race Health Observatory, 2024, p.7).

The Orthodox Jewish community is heterogeneous, consisting of several subgroups that differ in their observance level. Historically, these communities have resided in enclaves in Salford, Manchester and London boroughs of Hackney and Haringey (Staetsky, 2022). This living pattern and a high fertility rate amongst the Haredi subgroup, of seven children per woman (compared to the OECD average of 1.5) (Staetsky and Boyd, 2015; OECD, 2024), resulted in a few geographical areas in the UK where these communities are one of the largest maternity service users. For example, a local City and Hackney Council report estimated 22% of the children under 19 in the borough to be Haredi (Mayhew et al., 2011).

Despite policy efforts, some UK Jewish patients report barriers to accessing health services, a lack of cultural awareness within NHS services, and feeling judged and stereotyped (Hackney and City, 2018; Interlink, 2014; Letley et al., 2018; Phillips and Hakak, 2023; Wineberg and Mann, 2016). There are also reports of cultural and language barriers specific to Orthodox people (Wineberg and Mann 2016; Tkatch et al., 2014; NHS Race Health Observatory, 2024; Spitzer, 2019). This was echoed by a recent NHS report: *"current NHS communication practices do not align with the cultural and religious lifestyle of the strictly Orthodox community"* (NHS Race Health Observatory, 2024, p.12). The above comes in stark contrast to the NHS constitution, which pledges that every person in the UK to be treated equally, in a way that meets personal needs and preferences (Department of Health and Social Care, 2023). Understanding the religious and cultural needs of people from the Orthodox Jewish communities is essential to addressing this gap in care.

Orthodox Jews (OJ) adhere to the Halacha (Jewish law), a set of principles encompassing every aspect of daily life, including healthcare practices and women's health (Flint-Ashery, 2020). Some of the customs and restrictions have a direct impact on maternity care and the use of services. A central example is the observance of *Shabbat* (Saturday) and Jewish holy days, during which most daily activities, including writing and using electronic devices, are prohibited. Given the unpredictability of childbirth, when occurring on *Shabbat*, it may present complex challenges for the

birthing person and their family. For example, signing consent forms or using electric facilities (e.g., automatic doors or faucets) may be prohibited. During my PCAF, I spent a Shabbat with people from the community and joined a few ceremonies and customs to start gaining a better understanding of these restrictions. Dietary laws are another example; OJ individuals follow a strictly kosher diet, requiring meals prepared in kosher kitchens, using kosher food and kitchenware. Therefore, food provided by UK hospitals is strictly forbidden, and OJ couples will require pre-ordered sealed meals. A woman from my PPIE group shared that following her baby's transfer to the intensive care unit a couple of days after birth, she was not provided with any food, and since it was *Shabbat* she could not order or pay for delivery. Another Halachic restriction is the prohibition of physical contact between people of opposite sex outside the immediate family, unless deemed medically necessary (Spitzer, 2019). Immodest graphics or illustrations of the human body are also forbidden. The above can create challenges and tension when interacting with healthcare professionals, particularly around topics concerning women's health, due to its sensitive nature. Adding to these challenges is that for Haredi women, interactions with the NHS might be the first or only interaction with the world outside their community (Kasstan, 2019). This can contribute to communication barriers. For example, a PPIE member shared that her inability to engage in casual conversations with healthcare providers negatively impacted her care. Another woman described interactions with NHS staff as a "cultural shock." These experiences may stem from the separate educational system attended by most OJ, which is typically, as PPIE members indicated, single-gendered and does not include anatomy or sex education. The very young marriage age of women from certain Orthodox subgroups may add challenges to these interactions.

PPIE members also shared that OJ women may be reluctant to book their pregnancy by 12 weeks, as recommended, due to cultural conceptions. Poor adherence to health programmes is well documented. For example, several reports highlighted low uptake of national children vaccination programmes (Letley et al., 2018) and pregnancy screening tests (Kasstan, 2019; Simhi et al., 2020) amongst Haredi women. There is also evidence to suggest that some Haredi women will decline caesarean section due to religious and cultural conceptions (Spitzer, 2019; Bar-El, 2024).

In addition to cultural and communication barriers, digital exclusion may be a barrier for OJ in accessing care. While the NHS is moving towards digital services, low digital literacy amongst some patients can limit their access to health information. While this challenge has previously been reported for the Haredi community (Interlink, 2014; Hackney and City, 2018; Wineberg and Mann, 2016), it reflects a wider national issue, with 10% of adults in the UK identified as 'internet non-users' (Serafino, 2019). The paper-based MAMMA card that will be developed through this project will offer an accessible alternative, with the potential to be adjusted and adopted by other communities.

Lastly, OJ women may be facing various challenges related to their multi-layered identities, some of which are defined as protected characteristics under the Equality Act, such as gender, race, maternity and religion (*Equality Act,* 2010). Moving away from the "one size fits all" approach, the proposed study will examine these challenges through an intersectional lens (Kapilashram and Hankivsky, 2018) and help tailor services to meet the community's needs.

Overall research questions:

1. What information do OJ families wish to include in a MAMMA card, and what is their preferred design? In particular:

A) What are the unique needs and experiences of OJ women and their partners when interacting with NHS maternity services, and how are these currently being met by NHS services?
B) What can be done to increase the sense of belonging and ownership of Orthodox Jewish families to NHS maternity health promotion programmes?

2. What features and/or health promotion do NHS maternity staff wish to include in the MAMMA card?
3. What are the challenges and enablers that Orthodox Jewish women and NHS maternity staff face when working together, and how can this be addressed using the MAMMA card?

****Aims and objectives****

Overall aim: to increase satisfaction and adherence to health programmes for Orthodox Jewish families in the UK by improving communication and trust between these families and NHS maternity staff.

Overall objectives:

1. To understand the lived experiences of OJ couples when interacting with NHS maternity services
2. To understand what health promotion or other features NHS maternity staff want to include in the MAMMA card
3. To assess the feasibility of a new communication tool in the MHS maternity
4. To co-design a culturally sensitive maternity card

****Methodology / Plan****

The proposed study was designed as Community-Based Participatory Research (CBPR), consisting of three work packages (WP). Figure 1 illustrates the study outline and the PCAF input, adapted from the Action Research model (Kemmis et al., 2014). The WPs are described in detail below.

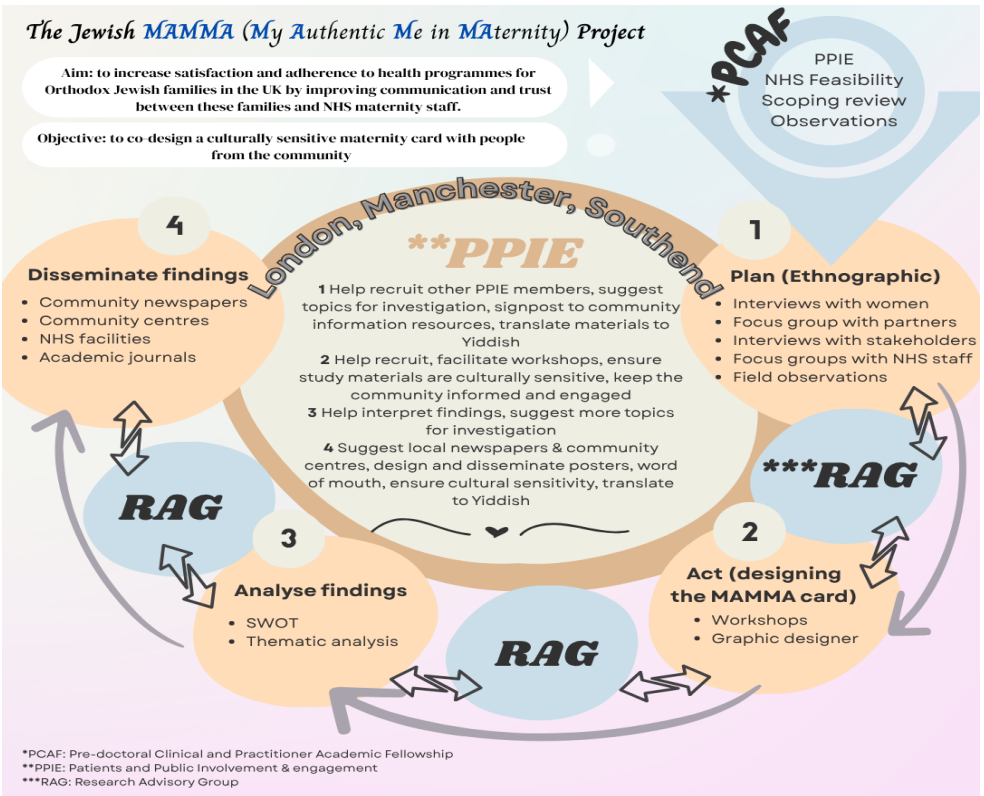

Figure 1: the MAMMA project outline, adapted from Kemmis et al. (2014)

Engaging marginalised communities in the design and implementation of health promotion programmes can reduce health inequalities and lead to better adherence to health recommendations (Morales-Garzón et al., 2023; Haldane, Chuah et al., 2019; MacDonald et al., 2024). People from the community will be involved in all stages of the project, including designing the maternity card that will be used by their communities, to enhance the feeling of ownership and ensure the content is culturally sensitive. Following the co-production of the prototype, the card could be replicated and tailored to other communities, adjusting the prefix to reflect each community. In this study, *the **Jewish** MAMMA Card*. There have already been attempts to create a maternity Muslim card (Chelsea and Westminster Maternity and Neonatal Voices Partnership, 2024), and the MAMMA card can be easily adapted for this purpose.
The project was designed with inclusivity in mind. The main aim of the study is to make NHS maternity services accessible and available for marginalised groups. In addition, during my PCAF I set the foundation for including Orthodox males, who, as indicated by my PPIE group, do not usually engage in research or share personal information. I will also make all study materials available in paper print, to account for low digital literacy and consider copies in Yiddish. I will take a Neurodiversity in Academia course to ensure active participation of neurodivergent individuals.

Frameworks

The following frameworks will be utilised throughout the research project:

- Intersectionality (Crenshaw, 2013) will support understanding how Orthodox Jewish families experience maternity care, considering their multilayered identity.
- Purity and Danger (Douglas, 2002) will help understanding Jewish practices and rituals, and how these can create conflicts with NHS facilities and practices.
- Pedagogy of the Oppressed (Freire, 1970) will guide the engagement with the community by creating dialogue, critical thinking and empowerment.
- The Positive Deviance approach will help identify the community's successful strategies (Marsh et al., 2004).

Groundwork

During my six-month PCAF programme, I enhanced my cultural competence and established rapport and trust with stakeholders from the community in London and Manchester, who will form the PPIE group, and introduced my project to City and Hackney Council and many NHS staff. In Homerton, I had discussions with the head of midwifery, QI lead, lead consultants (labour ward, scan department and obstetrics), Equity, Diversity & Inclusion lead, delivery suite and community matrons, clinical placement facilitator, screening midwife lead, and community midwives. I had meetings with R&D and community midwives in UCLH, Southend and Whittington to assess feasibility. All were very supportive of the project and the card concept.

Work package 1: interviews with community members and focus groups with NHS staff

Research questions

1. What are the unique needs of Orthodox Jewish couples that are using NHS maternity services, and are these needs being met?
2. How do Orthodox Jewish couples experience NHS maternity services?
3. What can be done to increase the sense of belonging and ownership of Orthodox Jewish families to NHS maternity health promotion programmes?

Aims and objectives

1. To gain an in-depth understanding of the lived experiences of OJ couples (service users) when interacting with NHS maternity services.
2. To assess the feasibility of implementing a communication tool to support NHS

maternity staff (end users) when interacting with Orthodox Jewish families.
3. To identify health topics that are most important to promote within the community.
4. To gain a better understanding of the role of community figures (e.g., doulas, rabbis) in mediating between NHS services and the Orthodox Jewish couples.
5. To assess the interaction between NHS maternity staff and Orthodox Jewish families and identify barriers and facilitators.

Design

I will employ a focused ethnography approach, as this is well-suited to time-limited health research involving a specific cultural group (Knoblauch, 2005).

Eligibility

- Eligible women will be pregnant at the time of interview or mothers of at least one baby that was born in the last year, to minimise recall bias. Their partners will be invited to the focus groups.
- NHS maternity staff working with the Orthodox community (doctors, midwives, midwifery students and maternity support workers).

Setting

Data will be collected from London, Manchester and Southend (the largest UK Orthodox communities), to allow for geographic diversity.

- Women will be interviewed in their homes, as advised by the PPIE group.
- Men will be invited to attend a focus group, as advised by the PPIE. The venue will be a Kosher place in the community, identified by the PPIE group.
- Maternity staff will be recruited from NHS trusts in London (Homerton, Whittington, UCLH), Manchester and Southend. Focus groups will be online, using Teams.

Data collection and sample

I will utilise purposive and snowball sampling to recruit Orthodox women (n=25-30), partners (n=15-20) and stakeholders, e.g., doulas, charity leaders, etc. (n=5-8), or until information power is achieved. The sample size is based on the focused aims of the project and the quality and diversity of recruitment, which would yield rich data (Malterud et al., 2016). My groundwork showed that recruiting men could be challenging and that women will provide richer data, therefore, the men's sample size is smaller. To ensure diversity of participants, recruitment will include people from different Orthodox sects, different geographical areas, different ages, family sizes (e.g., primiparous and grand multiparous) and sociodemographic backgrounds. I will distribute infographics and leaflets in the NHS and community facilities to maximise recruitment. Figure 2 is a poster example that was designed with the help of the PPIE lead to ensure appropriateness. The Hebrew text translates to "congratulations on your pregnancy" and was chosen to enhance trust and ownership.

I will also distribute emails to NHS maternity units, calling staff to participate, and recruit an estimated total of 15-20 staff. NHS staff will include students and junior and senior staff, from different ethnicities and levels of experience.

I will conduct:

- Semi-structured in-depth interviews lasting around 60-90 minutes with women
- Two (in London and Manchester) up to 90-minute focus groups with men
- Two online focus groups with NHS staff
- Ten non-participant convenient observations in maternity units, depending on service capacity (e.g., antenatal clinics, triage).

Interviews and focus groups will be recorded with consent.

Analysis

- Thematic analysis will be utilised to identify themes from interviews with families and community stakeholders.

- Building on the Positive Deviance (PD) model (Marsh et al., 2004), positive behaviours or experiences (e.g., stories of excellent communication with staff) will be identified and coded.
- I will explore the use of the SWOT model to identify strengths, weaknesses, opportunities and threats in the interaction between service users and NHS staff.

Insider outsider reflection notes

I will utilise Merton's insider-outsider proposition (Merton, 1972) as a secular Jewish woman and as both a service user and an end-user (a midwife) to write reflective notes. This will allow a focused investigation that has the potential of leading to richer data, as Merton stated: *"one must not only be one in order to understand one; one must be one in order to understand what is most worth understanding"* (Merton ,1972).

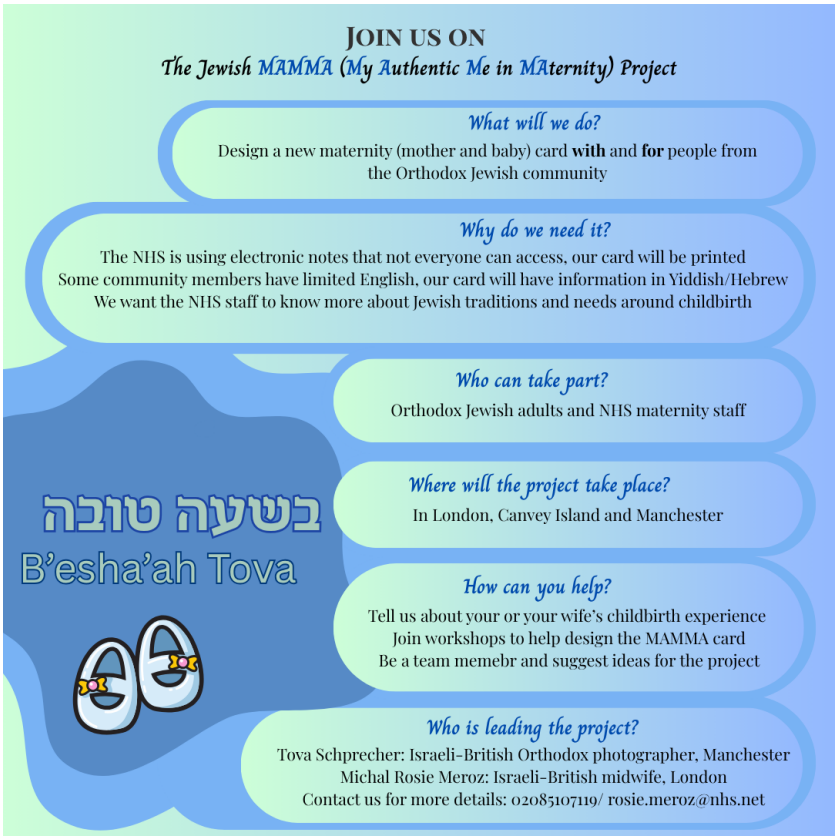

Figure 2: the MAMMA project, a recruitment poster example

<u>Work package 2: workshops with Orthodox women and NHS maternity staff, to initiate the development of the *Jewish MAMMA Card*</u>

Research questions

- What elements should be included in a tool that aims to improve communication between NHS maternity staff and Orthodox Jewish communities in the UK?
- What features do NHS maternity staff wish to include in the MAMMA card?
- What is the preferred design of the MAMMA card?
- What are the challenges and enablers that Orthodox Jewish women and NHS maternity staff face when working together?

Aims and objectives

Bringing together Orthodox Jewish women, community stakeholders and NHS maternity staff, and guiding them to co-produce the MAMMA card.

Design

The workshops will be structured and facilitated by the PPIE and NHS leads. Utilising the Positive Deviance model (Marsh et al., 2004), "success" stories described in WP1 will be shared and provide the basis for positive discussions. I will join the workshops as a non-participant and will take field notes. The PPIE lead will use her photography skills to document the workshops and create a photographic storytelling of her experience. We will work together towards creatively publishing this.

In a group: introducing the facilitators and participants. The facilitators will discuss the study, the shared goals and the plan for the day. Code of conduct and confidentiality will be agreed; participants will sign a consent form a-priori. The themes identified during WP1 will be presented to the participants and written on a board, to allow a visual reminder that will guide the workshops.

In pairs: each Orthodox participant will be paired with an NHS staff member. The facilitators will provide each pair with two themes, identified during WP1. The pair will discuss the themes and agree on a written description that is negotiated and agreed on by both sides (both medically correct and culturally sensitive). Two of these themes could be, for example: what are the Shabbos restrictions during childbirth, what is vitamin K and its importance for newborns.

In group: couples will present their work, facilitators will close the session, providing participants with information regarding the next stages and engagement options.

Eligibility

- Orthodox women with previous experience of NHS maternity services.
- NHS maternity staff working with the Orthodox community (doctors, midwives, midwifery students and maternity support workers).

Setting

The workshops will take place in community venues in London and Manchester, identified by PPIE members during my PCAF. Kosher food will be served to attendees. The venues are family-friendly, and women can bring their young children with them. Childcare coverage will be offered to women who require it.

Data collection and sample

Orthodox women and stakeholders (n=10-15) and NHS maternity staff (n=10-15) will be invited to participate in each workshop (Total n=40-60). Sampling will be purposive, to allow diversity among women and staff. Women and staff from WP1 will be invited to join the groups and, through snowball sampling, will be encouraged to help recruit other participants. PPIE work reassured me that this recruitment strategy and sample size are feasible. An Orthodox graphic designer, identified during my PCAF, will join the workshops to gain a better understanding of the design requirements.

To mitigate power imbalances, venues will be selected by the community to ensure appropriateness. Workshops will be co-facilitated by NHS and community representatives, with community members shaping the workshop content and agreeing on ground rules at the outset. A Yiddish/Hebrew interpreter will be present to support inclusivity. Freire's model of dialogical learning and the Cultural Humility approach will be followed to ensure respectful collaboration and open dialogue (Freire, 1970; Tervalon and Murray-García, 1998).

Safety considerations

I will complete a City St George's risk assessment form to help identify any potential risks involved in participation, minimise them and refer as appropriate. Potential risks could be related to the environmental hazards (e.g., electricity), physical condition (e.g., pregnancy) or emotional distress. Please see the Ethics section for more details.

Analysis

Following the workshops, women and NHS staff will be encouraged to write reflective notes that will be analysed. I will utilise Narrative Inquiry (Clandinin and Connelly, 2000) to explore participants' experiences, which will lead to publication. The PPIE lead, who has extensive experience working with the community, will create a photographic storytelling of participants' experiences, with my support. Women will sign a consent form a priori, and those who decline will be excluded from any recording.

I will consider utilising the SWOT model to identify strengths, weaknesses, opportunities and threats involved in the interaction between service users and NHS staff.

Work package 3: designing the MAMMA Card

Stage 1

The PPIE lead, the graphic designer and I will review the topics agreed on during WP2 and develop the *Jewish **MAMMA (M**y **A**uthentic **M**e in **MA**ternity) card*. The content and design will be appropriate; for example, there will be no graphic illustrations, as they are forbidden. Positive Deviance will guide this stage (Marsh et al., 2004), and we will look at positive experiences shared during WP1 and observed during WP2 to highlight and suggest solutions that stem from successful solutions that already exist. The content of the card will be decided by the participants during WP2, however, the PPIE work during my PCAF suggested that it may include the following:

1. A list of Jewish laws that are relevant during pregnancy and childbirth, and the main needs of Orthodox couples when interacting with NHS maternity services.
2. Health promotion that will be written in simple English and Yiddish.

Stage 2

The ***MAMMA card*** will be presented to the Research Advisory Group (RAG) and the PPIE group. RAG will consist of leading NHS and community figures who have been identified during my PCAF. Throughout the project, the group will meet biannually and at this stage, we will discuss the content and design and suggest amendments. PPIE feedback will also be sought.

Setting

The RAG will meet in Manchester and Homerton hospitals, and an option for an online meeting will be considered, as long as no women are excluded.

Ethics and data storage

The study will adhere to GCP standards, and I will seek ethical approval as advised by my host and partner organisations. I will apply via IRAS to the HRA and NHS REC for ethics. PPIE leads who help with recruiting will undertake the NIHR GCP course. Information sheets (PISs) will be given to all participants before any involvement. These will specify the aims and outline of the project, the duration and nature of involvement and the voluntary aspect of taking part, emphasising that consent can be withdrawn at any stage. Participants will be asked to sign a consent form. PIS and consent form will be available in English and Hebrew/Yiddish. Interviews and focus groups will be recorded on a GDPR-compliant encrypted Dictaphone. Once transcribed, data will be anonymised and stored on a password-protected computer, bought and used solely for the study and transferred to a secure university server. The password will be known to me and my lead supervisor only.

It is not anticipated that taking part in the study will induce distress, however, I will join interview training to learn sensitive and non-judgmental interviewing techniques. If a participant experiences stress or discomfort during any phase of the study, the

interview or participation in the workshop may be paused or stopped, and referral to appropriate support will be offered.

****Knowledge mobilisation, dissemination and impact****

Dissemination

Academic, community and professional disseminations will be embedded throughout the project, as follows:

- **WP1:** findings will be shared via an academic paper, NHS R&I meetings, City & Hackney Council and Homerton Board presentations and a plain English summary for community newspapers.
- **WP2:** a photographic storytelling co-produced with the PPIE lead will be displayed at Beis Brucha (Orthodox mother and baby centre), local media, and conferences. I will also publish reflective notes and a peer-reviewed article.
- **WP3:** introducing the Jewish MAMMA Card to NHS trusts in London and Manchester, meetings with NHS policymakers, and Orthodox community stakeholders. Presentations at the RCM Conference, the Jewish Congress (Jerusalem) and the Israeli Midwives Association Conference. A final academic paper will document the research project and design of the tool.

In addition, dissemination will be supported through NIHR networks, local Clinical Research Networks, and co-produced plain English materials led by the PPIE group. NHS training sessions will support the adoption and spread of the tool.

Intended outcomes and impact

Figure 3 illustrates the anticipated impact of the project.

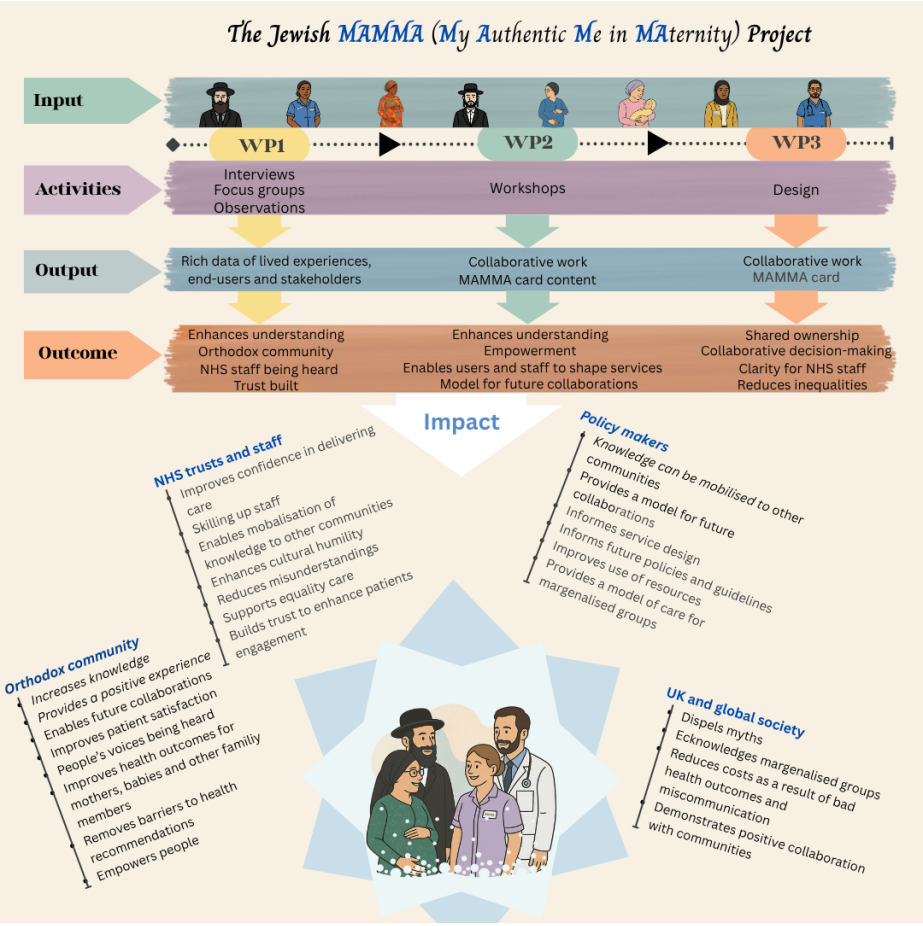

Figure 3: the MAMMA project outcomes and impact, images generated by ChatGPT (OpenAI, 2025)

Project management
My two PCAF supervisors will continue to support me during the PhD, joined by another internal supervisor, an external supervisor, and a named mentor. As with my PCAF, we will use a shared Microsoft Teams folder to store and collaboratively review sections of my work, and access to this folder will be password protected. I will also maintain a live Gantt chart to track progress and milestones, ensuring transparency and accountability across the team. We have agreed to hold biannual Teams meetings to formally review progress, supplemented by ad hoc meetings as needed. My work will also benefit from the range of university and school-level resources, training and support opportunities, which are aligned with the VITAE framework.
Please see Table 1 for success criteria.

Table 1: Success Criteria

Measurement	Risk	Contingencies to Deal with Risks
Recruitment and retention of PPIE and RAG members	• Hard recruitment due to large family obligations • RAG retention	• Clarity of commitment at the outset • Recruitment started during PCAF • Large recruitment to compensate for retention • Money incentives
Meeting recruitment targets for ethnographic study	• Potentially difficult to recruit participants from insular community • Language barrier	• Building rapport and trust with women during PCAF and interviews to allow for snowball sampling • Building rapport with community stakeholders • I speak Hebrew, the PPIE lead speaks Yiddish • Money incentives: thank you vouchers
Meeting recruitment targets for workshops	• Recruiting from insular community • Language barrier • Workshops might be too lengthy	• Women from interviews will be invited to workshops, PCAF work • Money compensation: thank you vouchers • Baby friendly facilities, kosher food • Consult PPIE for best times for workshops, in line with school hours and Jewish holidays
Delivering each project stage as per the timeline	Each stage is dependent on successful completion of the stage prior	• Groundwork during PCAF • Supervisors' advice • Enough time between stages • Women from interviews will be invited to workshops to allow for continuity
Successful of workshops	Lack of applicant experience	• Main applicant and two PPIE members will have facilitator training
Successful production of the tool	• Limited design experience • Accessibility	• Discussions with NHS managers and community key figures during PCAF • Graphic designer from the community • PPIE and RAG will approve content
Implementation	• Beyond PhD scope • NHS barriers	• Feasibility assessment during PCAF • Follow up project

Intellectual property and commercialisation
The proposed study will generate a non-commercial maternity card, the Jewish MAMMA Card. The primary aim of the card is to maximise public benefit by supporting NHS maternity services to deliver inclusive maternity care; therefore, the card will be made freely available for use across UK maternity services. The tool will be designed with adaptability in mind, allowing it to be used by other ethnic minority groups or communities across the UK, and potentially other countries where appropriate.

****Training courses****

Course name	Year	Provider	Justification
Basics of Halacha (Jewish laws)	1	TBC	To familiarise myself with the Jewish laws which dictate Orthodox Jewish life, including pregnancy and childbirth
Intro to Participatory Research	1	Social Research Association	Learn the methodology I will utilise in my study
Depth interviewing skills (online)	1	The National Centre for Social Research (NatCen)	Improve my interviewing skills and how to overcome challenges prior to starting the interviews of WP1
Effective Community Engagement	1	The Centre for Ethnic Health Research	Learn how to improve the community engagement, as part of this participatory study
Neurodiversity in Academia	1	Cambridge University	Upskill in inclusive research practices to ensure the participation of neurodivergent individuals
SAPPHIRE summer school	2	University of Leicester, Ethnography for healthcare	Learning about Ethnography research, designing, conducting, ethical considerations, analysis and publication
Participatory Action Research Course	2	UK Participatory Research network, Durham University	Improve the basic knowledge I gained in year 1
Introduction to Nvivo course	2	National Centre for Research Methods	Learn how to analyse qualitative data using Nvivo software
Qualitative Data Analysis	3	Social Research Association	Refresh and improve my knowledge on qualitative data analysis
Publication skills training	3	UK EQUATOR Centre Publication School	Improve my publication skills and learn new skills and requirement of journals

****Placements****

During my PCAF I identified and agreed on placements:

1. Consultant midwife at St Mary's trust, Manchester: year 2 or 3 for 2-3 weeks.
2. National Perinatal Epidemiology Unit (NPEU) in Oxford: year 4 for 2 weeks.
3. Jewish Policy Research (JPR), London: year 1 for a month
4. Homerton and St George's, and City R&D on a flexible basis
5. Tahareinu (see "Other").

****Building Collaborations****

I have established good collaborations with several Israeli qualitative researchers with expertise in health research; their location in Israel, where the largest Orthodox Jewish population resides, is of value. Professor Anat Gesser-Edelsburg, the head of the Health Promotion Programme, School of Public Health, Haifa, will act as my external supervisor. Haifa and Hackney have a twinning relationship. Professor Rivka Hillel-Lavian, head of the special education program at the Faculty of Education in Levinski College of Education and a leading expert in photovoice research, will support the study as needed. I also maintained my Israeli nursing and midwifery registration. I have a large network with people in key roles in maternity that I have contacted, and they are keen to collaborate and share their strategies for improving communication with the Orthodox community. I was invited to introduce my work during the annual event of the Israeli Midwives Association.

I have built collaborations at Homerton Hospital with midwives, doctors and patient advocates. I communicate regularly with all the maternity ward matrons, the Equality, Diversity, and Inclusion (EDI) midwife, the audit midwife, and consultants. During my PCAF, I ran drop-in sessions to introduce the Orthodox Jewish community and my project; joining me on these sessions were the City & Hackney public health team, Homerton junior and senior midwives, doctors, consultants, QI leaders, Whittington R&I team, and NHS Orthodox Jewish staff from different trusts.

I have developed informal relationships with senior researchers, research groups and leaders who provide a strong network of mentors and collaborators. This includes chief investigators of some of the studies we undertake at Homerton, as well as a few research midwives who applied and won the PCAF and DCAF awards. I have established the Academic Midwives Network, where we share and support each other. During my placement at City University, I engaged with the midwifery academic staff and was invited to conduct a workshop with midwifery students on the topic of research with ethnic minority groups.

I established a collaborative relationship with people from the Orthodox Jewish community, some are charity volunteers and leaders, such as Rachel Klien, the HJMT founder and director, a charity that provides support during childbirth to Jewish mothers. I also communicate and collaborate with Interlink, a Jewish charity that brings together Charedi community organisations operating in maternity, and statutory providers and commissioners, with a formal reporting structure back into the sector. I met with a few religious leaders, one of them is rabbi Grunfeld from MARS, a UK Jewish organisation that helps advocate for Orthodox families and support them with health choices. I made a strong connection with Mrs. Sara Neiman, an Orthodox Jewish woman from Stamford Hill and the director of a mother's health charity. I visited Beis Brucha, an Orthodox mother-and-baby centre in London, and met with the manager, who advised that any promotional material can be presented on their digital screens. I also joined Homerton's community midwife on site to assess recruitment feasibility and found the staff and women to be very receptive. I joined Shifra, a Jewish maternity staff group that educates and advocates for Jewish women and staff.

I contacted the Research Design Service, later the Research Support Service, and received their support.

During my PCAF, I met with the Whittington R&D team and informed them of my project, and they were happy to support. I also visited a few other trusts that provide care to large Orthodox Jewish communities, such as UCLH and Manchester and started building relationships with the staff there.
My academic supervisors will keep supporting me in my academic career, as well as helping me extend my professional collaborations.

****Conferences****

Conference	Location	Year	Rationale
Action research and participatory methods	school of education, Nottingham	2,4	Networking with like minded researchers, learn about new participatory studies, new methods and engagement techniques
The World Congress of Jewish Studies	Jerusalem, Israel	2,5	Engage with Jewish researchers and learn about studies on Judaism, challenges and solutions. Identify gaps. Present the project
The Race, Racism, and Health International Conference	UK	2,4	Learn about challenges and new policies related to race and health
RCM Research	location TBC	1,4	Maintain my professional registration, networking, present the project in year 5 with the PPIE lead
The Social Research Association annual conference	London, UK	3,5	Inform of new studies and networking with other researchers

****Other****
I will complete midwifery and Homerton's mandatory training annually, which amounts to approximately 60 hours per year. This includes, amongst others, safeguarding adults and children, inclusivity in maternity, midwifery skills, RCM and NHS updates and Oliver McGowan on Learning Disability & Autism. I will also update my GCP every two years.
I arranged a placement in Israel from October to March 2026. During my stay, I will take a Halacha course, meet my external supervisor and key figures in the health system that work closely with the Orthodox community and observe at trusts. I will also have a placement at Tahareinu, a charity that bridges gaps between medical innovations and the unique maternity needs of Orthodox families. I have discussed and agreed to this with the director, Rabbi Yizchok Melber, who has extensive connections within the global Orthodox community, and the Hasidic in particular, and is happy to support me. To sponsor my stay, I will be a guest lecturer, teaching inclusivity in maternity and research, at Haifa University, supported by my external supervisor, Prof. Gesser-Edelsburg.

CITY
ST GEORGE'S
UNIVERSITY OF LONDON

5th June 2025

Re: Michal Rosie Meroz, NIHR Doctoral Fellowship Application

I am writing in support for Michal Rosie Meroz (Rosie) in relation to her doctoral fellowship application.

City St George's, University of London (CSG) is a world leading University with an excellent reputation for research in Health and Medical Sciences and sector leading healthcare education, which also benefits from its location within the University of London and the North and South Thames ARCs. Rosie will be based with the Centre for Maternal and Child Health Research, within the School of Health and Medical Sciences. In the UK's last REF exercise, the Centre's impact case study was rated as 3* or 4* (internationally excellent/world leading) and submitted papers were rated as 3* (internationally excellent) and 4* (world leading), while the School's research environment was rated as 4*.

Within the Centre, we are keen to support Rosie to build on a very successful Pre-doctoral Clinical Academic Fellowship project through a Doctoral Research Fellowship. During the PCAF award, Rosie achieved very effective community engagement, establishing a very strong groundwork for the proposed project, and testing out its feasibility, focus and scope. She has completed the main work on a scoping review which confirms the research gaps in relation to this topic as well as providing a systematic understanding of existing knowledge. She has submitted the protocol for publication and is on track to submit the review itself in the next few months. These achievements clarify even further that there is a strong rationale for this project, that it is feasible and that Rosie has the potential to lead such a project and achieve an excellent doctoral outcome. Rosie's proposed project fits well with ongoing research projects in our Centre, which have a strong focus on equity of care access and experience and include a strong component of theoretically informed but practice relevant research.

Rosie has already proven academic capabilities, clinical experience and insights into research via her NHS role, in addition to her PCAF work, making the case for a strong future doctoral fellowship collaboration. Rosie shows excellent prospects as a future clinical academic and is likely to benefit hugely from a doctoral award, with a feasible and well-prepared project well attuned to doctoral studies and with a clear pathway to impact on care. She has in place an excellent network of academic support through Professor McCourt as lead supervisor, Dr Carol Rivas as external supervisor and consultant midwife Kylie Watson as clinical mentor as well as access to additional skills and experience within the Centre and School and support from an external advisor at the University of Haifa. Her CSG lead supervisor, Prof McCourt has very extensive research management and conduct experience in relevant fields as well as a strong record of research outputs and research supervision (31 students supervised to completion) and examining while her CSG co-supervisor, Dr Marina Daniele, is actively engaged in research on addressing inequalities in access to care and in doctoral supervision on relevant subjects such as improving migrant maternity experience, care for women with FGM and postnatal care for Black British mothers. Professor McCourt also has a strong track record of supporting career progression through post-doctoral awards, including obtaining and supporting Leverhulme and British Academy awards, NIHR Knowledge Mobilisation Fellowship and progression of doctoral candidates and postdoctoral

fellows to research fellowship posts within CSG and other universities, nationally and internationally and lectureships. She also has experience of supervising and supporting excellent MSc students in maternal health or midwifery to doctoral awards and of graduates to leadership roles in practice, such as consultant midwife, practice development, lectureship and policy advisor positions, including, for example, heads of midwifery schools internationally and senior advisor positions in the International Confederation of Midwives.

During her fellowship Rosie will be able to link with staff and collaborators with a range of expertise relevant to her work. The Centre for Maternal and Child Health Research comprises academic and dedicated research staff as well as doctoral and postdoctoral researchers from a range of clinical, social science and health science backgrounds. The Centre has an excellent track record in doctoral supervision, with a high rate of successful completions and with graduates progressing to academic and leadership positions. MSc, MRes and PhD students benefit from a strong peer group and a programme of activities such as weekly research seminars, themed reading groups, doctoral student conference, writing retreats and an early career researcher network. The Centre has established collaborative links with professional, voluntary and service user organisations, nationally and internationally as well as internal links with other relevant research centres and groups such as the Centre for Health service Improvement Research. This provides an excellent environment for Rosie to access a range of expertise and experience from academic and research staff as well as external collaborators in NHS and community organisations.

Research students benefit from a school-wide doctoral support and training programme and a programme of generic research training and support co-ordinated by the university's doctoral college, which is aligned with the Vitae framework. All doctoral students undertake initial and periodic skills assessments and are encouraged to design a bespoke generic training programme drawing on CSG provision, University of London and wider opportunities. Areas covered include subjects such as writing for publication and writing grant applications, as well as a range of personal, wellbeing and professional skills workshops and courses. All registered students are able to participate in relevant taught master's modules (e.g. advanced methods and data analysis, health management, educational practice) where appropriate, following discussion with their supervisors and relevant module leaders. Doctoral fellows have full access to this range of opportunities. Students also benefit from a strong strand of EDI activities and networks within the university and an extensive programme of support for early career researchers in health including an early career research network, funded opportunities such as pump priming and knowledge mobilisation awards and a suite of training and development workshops, mentorship programmes and study groups and themed networks.

Sincerely,

Dr Emma-Jane Berridge
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28 May 2025

To whom it may concern,

Re: letter of support for Michal Rosie Meroz, NIHR Doctoral Fellowship (DF) application

I am pleased to confirm that Homerton Healthcare NHS Foundation Trust will act as the partner organisation supporting Rosie during her NIHR Doctoral Fellowship. Ms Meroz is a senior research midwife at Homerton and has already completed a Pre-doctoral Clinical Academic Fellowship (PCAF), during which I acted as her mentor. She has consistently demonstrated a high level of commitment, integrity and leadership, particularly in undertaking research that addresses health inequalities in maternity care.

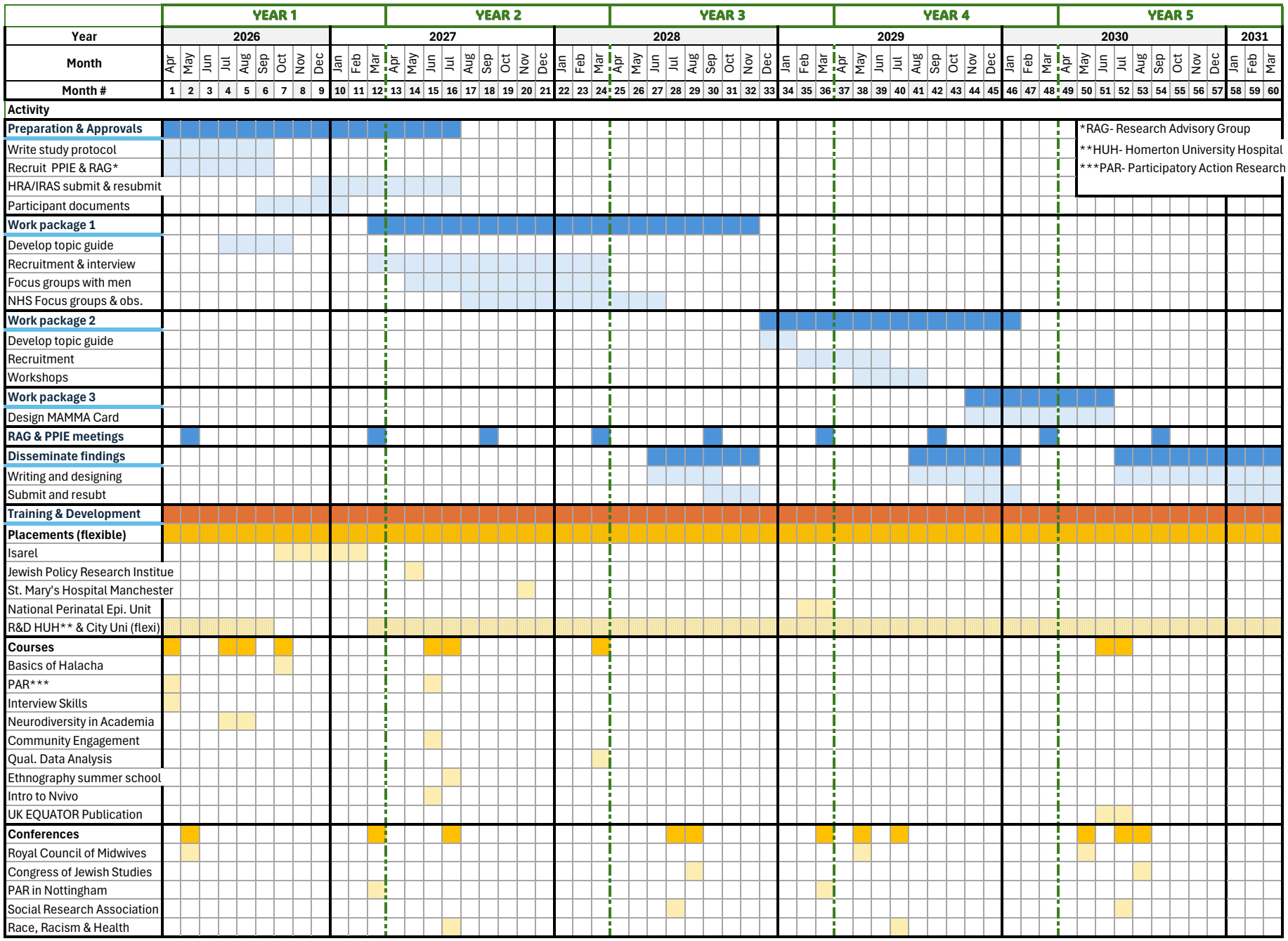
Homerton is uniquely positioned to support her proposed project, with nearly 20% of our birthing population identifying as Orthodox Jewish. While there are several ongoing community initiatives, there is a recognised lack of robust research in this area. Ms Meroz has successfully built trusted relationships with community members and maternity staff, carried out a feasibility assessment, and is well prepared to move into the doctoral phase.

We are committed to supporting her through this fellowship by providing administrative and governance support, and continued academic mentorship through our Research and Innovation department.

Homerton is committed to creating and maintaining an inclusive and supportive research environment. We actively promote equality, diversity and inclusion through recruitment, career development, and community engagement. We are proud to support Rosie's ongoing development as a clinical academic and believe that her work will make a valuable contribution to improving maternity care

Yours sincerely,

Mr H Malik
BSc (hons) MBBS PhD FRCS
Consultant UGI and General Surgeon
Homerton University Hospital
Director of Research and Innovation
Senior Visiting Fellow at London School of Economics and Politics
Humza Malik (lse.ac.uk)
www.linkedin.com/in/humza-malik-50642a59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Activity
Preparation & Approvals
Write study protocol
Recruit PPIE & RAG*
HRA/IRAS submit & resubmit
Participant documents
Work package 1
Develop topic guide
Recruitment & interview
Focus groups with men
NHS Focus groups & obs.
Work package 2
Develop topic guide
Recruitment
Workshops
Work package 3
Design MAMMA Card
RAG & PPIE meetings
Disseminate findings
Writing and designing
Submit and resubt
Training & Development
Placements (flexible)
Isarel
Jewish Policy Research Institue
St. Mary's Hospital Manchester
National Perinatal Epi. Unit
R&D HUH** & City Uni (flexi)
Courses
Basics of Halacha
PAR***
Interview Skills
Neurodiversity in Academia
Community Engagement
Qual. Data Analysis
Ethnography summer school
Intro to Nvivo
UK EQUATOR Publication
Conferences
Royal Council of Midwives
Congress of Jewish Studies
PAR in Nottingham
Social Research Association
Race, Racism & Health
*RAG- Research Advisory Group
**HUH- Homerton University Hospital
***PAR- Participatory Action Research

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